INDIVIDUAL CASE SAFETY REPORT (ICSR)

Final Project: Pharmacovigilance Assessment | Role: Drug Safety Associate

Report type	Spontaneous, initial report	**Report date**	12/10/2026
Prepared by	Sudipa, Pharmacy Student Intern, SNU	**Case ID**	ICSR-TASK5-001

DELIVERABLE 01: ADR IDENTIFICATION

Suspected adverse reaction	Generalized itching (pruritus) and skin rash over the chest
Suspected drug	Amoxicillin 500 mg tablet
Reaction type	Type B (hypersensitivity / allergic) reaction to a penicillin-class antibiotic
System organ class	Skin and subcutaneous tissue disorders (preferred terms: Pruritus generalised, Rash)
Valid ICSR check	All four minimum criteria are met: (1) identifiable patient (R.P., 32-year-old male), (2) identifiable reporter (Sudipa), (3) suspect drug (amoxicillin), (4) adverse event (pruritus and rash). The case can be reported.

DELIVERABLE 02: REPORTING (ICSR)

A. Patient information

Patient initials	R.P.
Age / Gender	32 years / Male
Weight	65 kg
Diagnosis	Acute bacterial sinusitis
Medical history	No history of allergy. No family history of diabetes or hypertension.

B. Suspected medicine

Drug (generic)	Amoxicillin tablet
Dose / route / frequency	500 mg, oral, three times daily (TID) for five days
Indication	Acute bacterial sinusitis
Therapy dates	Started 01/03/2026, stopped 03/03/2026
Manufacturer / batch / expiry	Not known
Concomitant medicines	None reported

C. Adverse reaction

Reaction	Generalized itching and skin rashes over the chest
Date of onset	03/03/2026 (day 3 of therapy)
Treatment of reaction	Antihistaminics
Action taken with drug	Drug withdrawn on 03/03/2026
Dechallenge / rechallenge	Dechallenge positive. Rechallenge not done.

D. Outcome

Outcome	Recovered completely
Date of recovery	06/03/2026

E. Case narrative

A 32-year-old male (65 kg) with acute bacterial sinusitis and no history of allergy was prescribed Tab. Amoxicillin 500 mg three times daily for five days. He started the drug on 01/03/2026. On 03/03/2026 he developed generalized itching and skin rashes over the chest. The drug was stopped the same day and he was treated with antihistaminics. He recovered completely by 06/03/2026. No other medicines were reported. The reaction is considered probably related to amoxicillin.

DELIVERABLE 03: CAUSALITY ASSESSMENT

Criterion	Finding	Result
1. Temporal relationship	Rash appeared 3 days after starting amoxicillin, a plausible interval.	**Yes**
2. Dechallenge	Recovered after the drug was stopped (antihistaminics were also given).	**Positive**
3. Alternative causes	No other drug, disease or infection reported that could explain the rash.	**None**
4. Rechallenge	Not done, and not advisable after an allergic reaction.	**Not done**

Naranjo question	Answer	Score
Previous conclusive reports of this reaction?	Yes (well known with amoxicillin)	+1
Event appeared after the drug was given?	Yes	+2
Improved when the drug was stopped or an antagonist given?	Yes	+1
Reappeared on re-administration?	Not done	0
Alternative causes?	No (none reported)	+2
Placebo, drug level, dose-response, earlier similar reaction, objective proof	Not available	0
Total (5 to 8 = Probable)		**6**

WHO-UMC category	**PROBABLE / LIKELY.** Plausible timing, improvement on withdrawal and no other apparent cause. Not 'Certain' because rechallenge was not performed.

DELIVERABLE 04: CLASSIFICATION

Seriousness	**NON-SERIOUS.** No death, life threat, hospitalization, disability, congenital anomaly or other medically important condition.
Expectedness	**EXPECTED.** Rash and itching are listed effects of amoxicillin in the product information.
Severity	Mild. Resolved with antihistaminics within 3 days of withdrawal.
Preventability	Not preventable. The patient had no earlier history of drug allergy.
Reporting action	Non-serious and expected, so no expedited report is needed. Include in routine and periodic safety reporting.

CONCLUSION

This is a valid, non-serious, expected adverse drug reaction (skin rash and itching) probably caused by amoxicillin. The patient recovered completely after the drug was stopped. Amoxicillin and other penicillins should be avoided in future, and the allergy should be recorded in the patient's medical file.

Prepared by: Sudipa, Pharmacy Student Intern, SNU Signature: _____ Date: 12/10/2026